

TB NAAT negative (MTB not detected)

- If no TB symptoms and testing for TB using TB NAAT negative, consider TB unlikely. Continue to assess need for TPT → 89.
- If TB symptoms, manage symptom/s as on symptom page, especially if cough → 38, if weight loss → 23, if fever → 24. Continue to assess and manage according to HIV status:

HIV negative

- Review in 1 week:
 - If TB symptoms resolve: no further follow-up needed. Advise to return if symptoms recur. Assess need for TPT → 89.
 - If TB symptoms persist: send sputum for TB culture and DST and arrange chest x-ray, assess clinically and review as below. If chest x-ray unavailable, refer and follow up culture and DST results.

HIV positive

- At this visit, also:
- Send sputum for TB culture and DST.
 - Arrange chest x-ray, assess clinically and review as below.

Doctor to review the chest x-ray, where available, and clinically assess the patient:

Compare with previous chest x-rays, if available. Especially look for: upper lobe cavitation, any lung opacification in HIV positive patient, pleural effusion/s, hilar lymphadenopathy, miliary TB, pericardial effusion.

Chest x-ray abnormal

- Pleural or pericardial effusion
- If bilateral pleural effusions or pericardial effusion, refer.
 - If pleural effusion, aspirate fluid and send 2 samples:
 - If clear: request TB culture, DST, ADA and cell count.
 - If pus: request TB NAAT, microscopy, TB culture and DST. Refer same day.

No effusion

- If unsure, discuss/refer.
- If history of mining or working in construction, consider silicosis.

Chest x-ray normal

Assess clinically: does patient have persistent symptoms or signs suggestive of TB?

Yes

No

If not yet done, send sputum sample for TB culture and DST if able to produce sputum and assess clinically, looking for other causes.

TB disease unlikely

- If previous TB in the last 2 years with positive TB NAAT → 91.
- If *no* previous TB in the last 2 years, start DS-TB treatment → 93 if:
 - TB NAAT trace.
 - TB NAAT negative in HIV positive patient.
 - Chest x-ray suggestive of TB in a patient with persistent TB symptoms and signs.
- Consider extrapulmonary TB:
 - If abdominal pain, swelling, hepatosplenomegaly, or diarrhoea, refer for abdominal ultrasound.
 - If severe headache/s, refer for CT scan/lumbar puncture.
 - If back pain, arrange spinal x-ray or refer.
 - If lymph node ≥ 2cm, aspirate lymph node for TB microscopy and cytology → 25.

- Continue routine care.
- Assess eligibility for TPT → 89.
- If done, follow up TB culture and DST results below.
- Advise to return if symptoms develop.

Follow-up culture and DST results every 1-2 weeks until available and review:

Culture positive (MTB confirmed)

No resistance to rifampicin and INH detected
Diagnose DS-TB: start treatment → 94.

Resistance to INH only detected
Diagnose INH mono-resistant TB: start treatment → 95.

Resistance to rifampicin detected
Diagnose RR-TB: start or refer to start RR-TB treatment → 99.

Culture negative

TB unlikely

- If TB symptoms resolved, continue routine care. Advise to return if symptoms recur.
- If TB symptoms persist, refer.